DATE: July 11, 2025
SUBJECT: Allegations of Regulatory Noncompliance, Invalid Policy Implementation, and Constructive Adverse Actions at Phoenix Indian Medical Center (PIMC), Indian Health Service (IHS)

I. INTRODUCTION

This disclosure is submitted pursuant to **the Inspector General Act** and follows the closure of my prior complaint to the U.S. Office of Special Counsel (**OSC Case No. DI-25-001642**) without referral. I request that the HHS Office of Inspector General investigate serious and continuing violations of federal policy, governance obligations, and personnel rules at the **Phoenix Indian Medical Center (PIMC)**, part of the Indian Health Service (IHS).

The misconduct detailed herein is institutional in nature, implicates public health risk, and includes improper removal of official duties and responsibilities without process or documentation.

II. REGULATORY AND POLICY VIOLATIONS

1. **Invalid Policies – Failure to Follow Required Approval Procedures**
   As of June 2025, at least **47 clinical and operational policies** in effect at PIMC were implemented or renewed **without complying with the Hospital Policy Management Policy** then in force. Specifically, the required steps for review and approval were bypassed:
   - Policies were not properly routed through or reviewed by the designated **Policy & Procedures Officer** (PX3147).
   - Policies were not formally **ratified by the Governing Body**.

These deviations render the affected policies **procedurally invalid**. They are being used to govern patient care, risk management, and staff accountability without legal or regulatory standing. Multiple written warnings and memoranda issued by me to Executive Leadership and the Compliance Officer have been ignored.

2. **Absence of a Sentinel Event Reporting Policy**
   PIMC continues to operate without a compliant **Sentinel Event Reporting Policy**, in violation of **42 C.F.R. § 482.21** and **CMS Conditions of Participation**. Despite internal awareness of this regulatory deficiency, leadership has declined to adopt or promulgate a policy aligned with CMS definitions and incident escalation thresholds, undermining patient safety and required response standards.

3. **Noncompliance with Indian Health Manual – Part 1, Chapter 1 (TN 24-16)**
   The October 2024 **Transmittal Notice 24-16** requires IHS facilities to maintain:
   - A formally published policy inventory
   - Properly ratified Governing Body bylaws
   - Defined approval channels for facility-wide policy
   - Internal policy authorities consistent with national standards

PIMC remains out of compliance with these directives. The governing framework lacks ratified documents, approved procedures, and internal accountability mechanisms. My attempts to bring the facility into compliance were obstructed.

III. OBSTRUCTION AND CONSTRUCTIVE ADVERSE ACTIONS

1. **Obstruction of Officially Assigned Duties (PD PX3147)**
   As the appointed **Policy & Procedures Officer (GS-0610-12)** for PIMC, I am responsible for overseeing policy development, coordinating regulatory compliance, and supporting the Governing Body. Since early 2023, I have been:
   - Excluded from all policy review processes
   - Denied access to regulatory platforms
   - Prevented from performing required duties despite holding the formal position

These actions directly obstruct the execution of my federal position and compromise oversight of safety-critical operations.

2. **Constructive Suspension**
   I was placed on **administrative leave** without charges on **June 27, 2025**, following protected disclosures to OSC, CMS, GAO, and Congressional offices. On **July 11, 2025**, I was **verbally informed** that I had been transitioned to **investigative leave for 30 calendar days**.
   - I have received **no written notice**, **no proposed charges**, and **no opportunity to respond**
   - I remain entirely removed from duty, systems, staff, and assignments

These facts support a claim of **constructive suspension** under **5 U.S.C. § 7512(2)** and **MSPB precedent** (e.g., *Aguzie v. OPM*, *Brown v. DOI*).

3. **Constructive Demotion**
   Although my grade and pay remain unchanged, the **removal of all substantive duties associated with PD PX3147**, exclusion from all policy matters, and lack of access to perform core functions constitute a **constructive demotion** under **5 U.S.C. § 7512(3)**. No formal notice or process accompanied this change in function.

IV. TIMELINE OF EVENTS

- **March 21, 2023**: Filed sexual harassment complaint with IHS HQ → placed on administrative leave within hours
- **June 25–26, 2025**: Filed protected disclosures with OSC, CMS, GAO, HHS OIG, and members of Congress
- **June 27, 2025**: Placed on involuntary administrative leave without charges or due process
- **July 11, 2025**: Verbally notified of transfer to **30-day investigative leave** without written charges, proposals, or procedural rights

V. REQUEST FOR ACTION

I respectfully request that the HHS Office of Inspector General:

1. **Investigate** the improper implementation of invalid policies without procedural approval
2. **Examine** PIMC's failure to adopt required sentinel event reporting mechanisms
3. **Evaluate** the obstruction of my official responsibilities under PD PX3147
4. **Review** the lack of due process in placing me on investigative leave and stripping duties
5. **Refer** findings to CMS, the Departmental Appeals Board, or enforcement bodies as necessary to restore regulatory compliance and institutional accountability

VI. ATTACHMENTS

- Position Description: PX3147 – Nurse Consultant, Policy & Procedures Officer
- Disclosure & Retaliation Timeline (March 2023–July 2025)
- July 11, 2025 Verbal Notification of Investigative Leave (Declaration)

- OSC Referral Closure Letter – DI-25-001642

Respectfully submitted,
/s/ Kevin D. Turnage
Dated: July 11, 2025

ATTACHMENTS

Declaration of Kevin D. Turnage

RE: Verbal Notification of Placement on Investigative Leave

I, **Kevin D. Turnage**, declare under penalty of perjury that the following is true and correct to the best of my knowledge:

1. I am employed by the Indian Health Service (IHS), U.S. Department of Health and Human Services (HHS), and I serve in the official capacity of **Policy and Procedures Officer**, GS-0610-12, under **Position Description PX3147** at **Phoenix Indian Medical Center (PIMC)**.
2. On **June 27, 2025**, I was placed on **administrative leave** without a notice of proposed action, charge letter, or any documentation supporting the removal from duty.
3. On the morning of **July 11, 2025**, I received a **verbal notification** from an agency official—acting under the authority of the PIMC Executive Leadership Team—that my administrative leave status had been formally changed to **"investigative leave"** for a duration of **30 calendar days**, effective immediately.
4. As of the date of this declaration, I have **not received any written documentation** explaining the basis for the action, outlining the nature of the investigation, or affording me any due process protections under **5 U.S.C. § 7513** or related authority.
5. I have been fully excluded from all duties associated with my position, denied access to government systems and files, and barred from performing any aspect of my official responsibilities—effectively placing me in a **non-duty status** for over 10 consecutive business days.
6. I assert that the agency's failure to provide formal notification, explanation of charges, or opportunity to respond constitutes a **constructive suspension** within the meaning of **5 U.S.C. § 7512(2)**, and that the agency has also effectively implemented a **constructive demotion** by stripping me of my assigned responsibilities without lawful process.

I affirm under penalty of perjury under the laws of the United States that the foregoing is true and correct.

Executed on this 11th day of July 2025.

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Kevin D. Turnage
1003 E Pinto Ct
Gilbert, AZ 85296
kdturnage@protonmail.com
901-220-8664